جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ايرينى واصف ابراهيم واصف
المحافظة : القاهره **الحي / المدينة :** خمسة عشر مايو **العنوان :** القاهره
رقم البطاقة : 28708152302281 **رقم التليفون :** 01184975958 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 15-08-1987 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايرينى واصف ابراهيم واصف
National ID: 28708152302281

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ايرينى
واصف ابراهيم واصف
مجاورة ١٨ - عمارة ٢ - مجموعة ٧
خمسة عشر مايو - القاهره

٢٨٧٠٨١٥٢٣٠٢٢٨١

١٩٨٧/٠٨

LQ7766089



٢٨٧٠٨١٥٢٣٠٢٢٨١ ٢٠٢٦/٠٢
حاصله على بكالوريوس خدمه اجتماعيه

انثى مسيحية متزوجة
ميشيل عريان عبدالسيد اسكندر
١ البطاقة سارية حتى ٢٠٣٣/٠٢/٢٧

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مستشفى روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

صحة مصر

03/24/2026

MDT FORM

Name:	Irene Wassef Ibrahem Wassef	**MRN:**	3695
Age:	39 Y	**ID:**	28708152302281
Menopausal Sta	POST	**Phone:**	01067358460
NRS SCORE:	0	**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

Sonomammography:

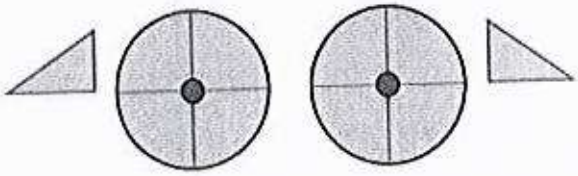
Mets Workup:

History:
STAGE 3 CANCER OVARY UNDERGONE TAH BSO 11/11/2025 HIGH GRADE SEROUS CARCINOMA BOTH OVARIES WERE INFILTRATED POSITIVE OMENTAL DEPOSITS PT IS ON ADJUVANT TAXOL CARBOPLATIN ,.BRCA 1 MUTANT FOR MDT CONCERNING NEED FOR OLAPARIB

Recommendations:

MDT Outcome & Plan
To add maintenance olaparib x 2 years (if no PD on platinum chemotherapy)
From NGO

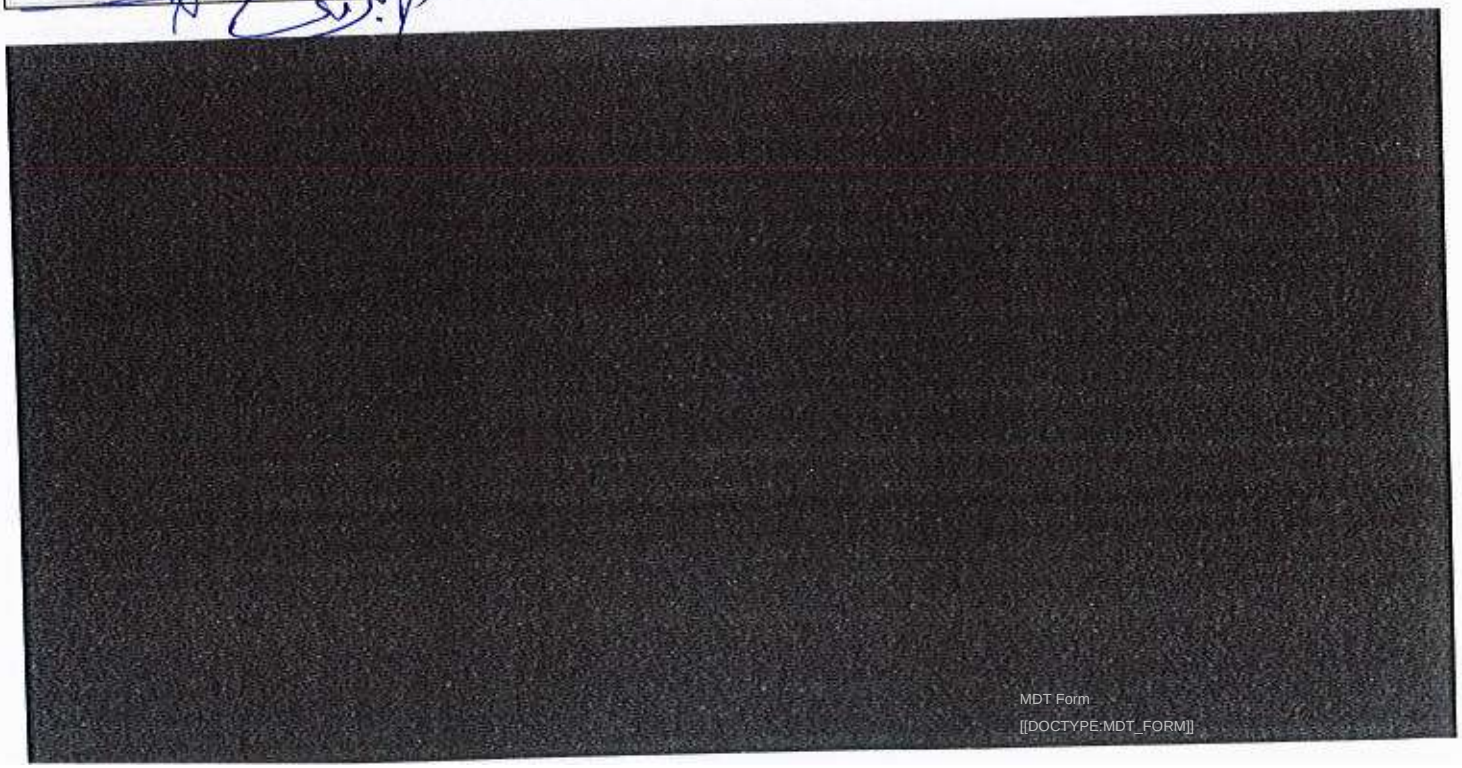
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد الشرقي				



Name: ايرينى واصف ابراهيم واصف **MR. :** 3695	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 38-year-old with stage 3 high-grade serous ovarian carcinoma who underwent a total abdominal hysterectomy and bilateral salpingo-oophorectomy on 11-11-2025. Post-operative chemotherapy was initiated with weekly Taxol 130 mg plus carboplatin AUC 2. The patient completed the first four cycles (W1–W4) before neutropenia prompted a one-week delay and consideration of prophylactic G-CSF. Subsequent cycles were administered as follows: W5–W6 with Neupogen 1 amp; W7 with Taxol 130 mg plus carboplatin AUC 2 and prophylactic Neupogen every two weeks; W8–W9 with the same regimen and ANC 3.65 × 10^9/L; W10–W12 with continued Taxol 130 mg plus carboplatin AUC 2; W13–W15 with the same regimen and monitoring of liver function, with sylimarine continued if hepatic enzymes remained elevated; W16–W18 with Taxol 130 mg plus carboplatin AUC 2, prophylactic Neupogen, and a plan to initiate olaparib for two years based on BRCA1 mutation status. The patient has also been receiving supportive care, including controloc and cetal at discharge, and has declined gastric prophylaxis or anti-emetics for liver support. Recent imaging on 27-06-2026 showed a few tiny pulmonary nodules and an L1 vertebral bony lesion; bone scan findings on 23-07-2026 indicated a small LV1 lesion likely a hemangioma rather than metastatic disease. The patient remains under consideration for ongoing olaparib therapy.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

detailed-medical-report
[[DOCTYPE:OTHER_DETAILED_MEDICAL_REPORT]]

Patient Name	: الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 03/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.
Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:
Thirty referred blocks labelled (1008099). Together with, thirty-five referred (H&E) stained slides.

Microscopic Examination:
* Procedure: Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration.
- Primary Tumor Site: Both ovaries.
- Laterality: Bilateral.
- Histologic Grade: High grade.
- Histologic Type: High-grade carcinoma, likely serous subtype.
- Description: Sheets, acini & papillae lined by highly pleomorphic malignant epithelial cells showing hyperchromatic nuclei.
- Mitosis: 20/10 Hpfs.
- Necrosis: Focal.
- Angio-invasion: Present.
- Ovarian Surface Involvement: Identified.
- Fallopian Tubes Involvement: Both tubes are involved from outside inward.
- Uterine body: Free.
- Cervix: Free.
- Vaginal cuff: Free.
- Appendix: Infiltrated by tumor.
- Omentum & peritoneum: Positive deposits of tumor nodules.
- Cytology: Positive for malignant cells.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 03/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:
Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):
- High grade carcinoma, likely serous carcinoma, involving (Both ovaries, both tubes, appendix, peritoneium & omentum).
- Free uterine body, cervix & vaginal cuff.
- Cytology: Positive for malignant cells.

* STAGE: pT3b.
* Immunophenotyping for (PAX-8, P53, ER, ...) is recommended.

Signature:
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.
Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:
Thirty referred blocks labelled (1008099).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; PAX8 clone (MRQ-50), P53 clone (Bp53-11) & ER clone (SP1).

Results:
* IPT (Positive control showed appropriate reactivity):
- PAX-8: Shows diffuse strong positive nuclear staining in the tumor cells.
- P53: Strong diffuse positive staining, Mutant.
- ER: Weak patchy positive staining in tumor cells.

Diagnosis:
Right ovarian mass with massive pelvic peritoneal nodules, Referred blocks (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):
The immunohistochemical findings (Positive PAX-8, Mutant P53 & Weakly positive ER) are in keeping with high grade serous carcinoma of mullerian origin.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايرينى واصف ابراهيم واصف
Analysis ID : 1008099
Age : 38 Y : 2 M : 2 D
Gender : Female
Referred By Prof. Dr. : فؤاد عبد الشهيد
Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules
Nature of Specimen : Total abdominal hysterectomy & bilateral salpingeo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration
Receiving Date : 2025-11-11
Patient ID : 743010

PATHOLOGY REPORT

Gross :

Three designated containers were received as:
1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy specimen,
Uterus measuring 6.5x6x3 cm. Endometrial thickness 0.3 cm, posterior outer surface of the uterus showed multiple nodules, the largest measured 2x2x1.5 cm. Myometrial thickness 2 cm and showed trabeculations.
Cervix 4 cm long, 3.5 cm in diameter with rubbery grayish white cut section.
Attached vaginal cuff was 1.5 cm.
RT ovary measuring 3.5x2.5X1 cm, ovarian mass measured 4x2x1.5 cm, cut section was greyish white rubbery causing disruption of ovarian capsule, right tube measured 6 cm long, showed fairly defined mass measured 12x7x4 cm, cut section was varigated greyish white yellowish.
LT ovary measuring 3x2x1 cm, ovarian mass measured 2.5x2x1 cm, cut section was greyish white rubbery causing disruption of the ovarian capsule, left tube measured 7 cm long.
Also received in the same container, Appendicectomy specimen measured 6 cm long, with 0.6 cm in diameter, maximal wall thickness was 0.3 cm, outer surface was glistening showing multiple nodules measured up to 0.8x0.8 cm.
2) Omentectomy: Specimen measured 42x8 cm, showing multiple firm nodules measured up to 1.8x1.8 cm.
Also received in the same container multiple peritoneal nodules measured up to 2x2 cm.
3) 16 cc of reddish fluid.

Microscopic :

Sections examined from the specimen (1) received revealed both ovaries showing infiltrating tumour formed of malignant epithelial cells with moderate to marked nuclear pleomorphism with scattered mitotic activity arranged in solid sheets, acini and papillae.
There is moderate desmoplasia.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايرينى واصف ابراهيم واصف
Analysis ID : [redacted] 1008099
Age : 38 Y : 2 M : 2 D
Gender : Female
Referred By Prof. Dr. : فؤاد عبد الشهيد
Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules
Nature of Specimen : Total abdominal hysterectomy & bilateral salpingeo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration
Receiving Date : 2025-11-11
Patient ID : 743010

PATHOLOGY REPORT

There is focal tumoural necrosis.
Ovarian capsule is disrupted.
Lympho- vascular space invasion is present.
Uterine wall revealed simple endometrial hyperplasia.
There is positive adherent peritoneal deposits.
Cervix is free.
Free vaginal cuff. The appendix is involved.
The tumor is adherent to outer surface of both tubes (right and left).
Sections examined from the specimen (2) received revealed omental tissue and peritoneal tissue positive for tumor deposits.
Cytological examination of the films and cell block prepared from the centrifugate of the fluid (3) received revealed eosinophilic proteinaceous material, red blood cells, lymphocytes, mesothelial cells and groups of cells with hyperchromatic nuclei.

Diagnosis :

Right ovarian mass with pelvic peritoneal nodules,
1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy & Appendicectomy, BILATERAL OVARIAN HIGH GRADE CARCINOMA LIKELY HIGH GRADE SEROUS CARCINOMA, INVOLVING BOTH TUBES FROM OUTSIDE, POSITIVE ADHERENT PERITONEUM, INVOLVED APPENDIX, FREE UTERINE WALL, FREE CERVIX, FREE VAGINAL CUFF.
2) Omentectomy with peritonectomy, POSITIVE FOR TUMOR DEPOSITS.
3) Aspiration, Cytology, POSITIVE FOR MALIGNANCY.
- TUMOR STAGE: pT3b.
- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

This report is electronically signed , any alteration or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جوستاف زوسي
النرناشونال

Name : Eriny Wassef Ibrahim Wassef
Branch : Portable
Ref.Physician Prof :

Date : 20-12-2025 11:14
Medical No. : GRI-133408
Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- No available history?! Metastatic work up.

Findings:

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

OPINION:

- **Free metastatic work up.**

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستافروسي انترناشونال

Name : Eriny Wassef Ibrahim Wassef
Branch : Portable
Ref.Physician Prof :

Date : 20-12-2025 11:14
Medical No. : GRI-133408
Ins.Company :

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Uterus could not be seen likely surgically removed.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No suspicious osseous lesions.

Opinion:

- **Free metastatic work up.**

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Dr. Mai Shaaban

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:MRI]]
M.R.I

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Irini Wasef Ibrahim Wasef
Branch : Maadi
Ref.Physician Prof : Goustav Roussey Egypt

Date : **09-07-2026 05:58**
Medical No. : **412963**
Ins.Company : تعاقد مستشفيات

NILE DIAGNOSTIC CENTER

MRI OF Lumbo-Sacral spine.

Techniques:

Sagittal and Axial T1 & T2 W Sequences

REPORT

- Normal alignment of the lumbar spine, with no signs of instability detected.
- ***LV1 small vertebral body lesion showed high T1 and high T2 likely small hemangioma rather than being suspicious for further bone scan assessment.***
- Normal marrow signal of the other lumbar vertebrae, with no other focal lesions identified.
- *LV5/SV1 small left posterior annular fissure is noted.*
- No significant disc bulges or herniations seen, with no thecal or foraminal compromise observed.
- Normal site, shape and intensity of the conus medullaris and cauda equina roots.
- No significant bony canal tightness seen.
- Normal pre and paravertebral soft tissue structures.

OPINION:

- ***LV1 small vertebral body lesion showed high T1 and high T2 likely small hemangioma rather than being suspicious for further bone scan /PET CT assessment.***

Dr.Reda Tag

Patient's Name:	**Irene Wasef Ibrahim Wasef**
Date:	**2026-06-06**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status post TAH-BSO for neoplastic lesion.

I- CHEST:

- Few tiny bilateral scattered pulmonary nodules largest measuring 5 mm at the right middle lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Status post TAH-BSO showing clear operative bed.
- Mildly enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Left internal iliac lymph node measuring 13 x 10 mm.
- No ascites.
- L1 vertebral body small lytic focus.

Opinion:

- **Few tiny bilateral scattered pulmonary nodules for follow up.**
- **L1 vertebral body small lytic focus for bone scan.**
- **PET-CT is advised.**

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]



National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]

Patient's Name:	ERINYWASEF
Date:	2026-04-08
Referred by:	Prof.Dr.
Hospital:	Goustave Roussey International Egypt

٣٦٩٥

جوستاف روسي

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 38-year old female with negative family history and pathologically proven ovarian cancer , screening study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance.
- Multiple circumscribed oval parallel homogenous hypoechoic benign looking soldi mass
- No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast parechyma with mottled appearance yet with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): breast adenosis.

Left breast (BIRADS 2): breast adenosis, for hormonal assay.

Best Regards,

Dr,. Amal Hegazi, M.Sc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

رئيس مجلس الإدارة
أ. د. حرمته كامل

مدير المعامل
أ. د. هند الشربيني

اسم الفرع : داون
مدير الفرع : د. نصرين ابراهيم
مدير الوحدة : أ.د عبد العزيز النقلي
طبيب الوحدة : د.ر علا السيسي
د.ر ايلت ابراهيم

المختبر
al mokhtabar
mpamera kamel lab

PATIENT NAME			Registered	26-10-2025 16:23:05	Collected	26-10-2025 16:31:08
ايريني واصف ابراهيم			Authenticated	27-10-2025 00:48:46	Printed	27-10-2025 07:18:52
Visit Number	Age	Gender	Referred By		Client ID	
20725545005	38 Year	Female	Prof : الدكتور رهيب		1618	

Test Name	Result	Unit	Reference Range	Previous Result
Kidney Function Tests				
Serum Creatinine	0.72	mg/dL	0.5 - 1.1	
Serum Urea	22	mg/dL	13 - 43	
Tumor Markers				
Alpha-fetoprotein (AFP)	2.0	ng/mL	Up to 8	
Assay Method: CLIA/ECLIA Results determined by using different methods may not be comparable.				
Carcinoembryonic Antigen (CEA)	0.67	ng/mL	Non smoker: up to 3.0 Smoker: up to 5.0	
Assay Method: CLIA/ECLIA Results determined by using different methods may not be comparable.				
CA 125	↑ 740.00	U/mL	0 - 35	
Follow up is recommended Assay Method: CLIA/ECLIA Results determined by using different methods may not be comparable.				

Dr.ABD El-Aziz El-Nokaly
Professor of Medical Bio-Chemistry,
Faculty of Medicine, Al Azhar University

ISO 15189 Medical Laboratories
CAP
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 03/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.
Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:
Thirty referred blocks labelled (1008099). Together with, thirty-five referred (H&E) stained slides.

Microscopic Examination:
* Procedure: Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration.
- Primary Tumor Site: Both ovaries.
- Laterality: Bilateral.
- Histologic Grade: High grade.
- Histologic Type: High-grade carcinoma, likely serous subtype.
- Description: Sheets, acini & papillae lined by highly pleomorphic malignant epithelial cells showing hyperchromatic nuclei.
- Mitosis: 20/10 Hpfs.
- Necrosis: Focal.
- Angio-invasion: Present.
- Ovarian Surface Involvement: Identified.
- Fallopian Tubes Involvement: Both tubes are involved from outside inward.
- Uterine body: Free.
- Cervix: Free.
- Vaginal cuff: Free.
- Appendix: Infiltrated by tumor.
- Omentum & peritoneum: Positive deposits of tumor nodules.
- Cytology: Positive for malignant cells.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايريني واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 03/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:
Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):
- High grade carcinoma, likely serous carcinoma, involving (Both ovaries, both tubes, appendix, peritoneium & omentum).
- Free uterine body, cervix & vaginal cuff.
- Cytology: Positive for malignant cells.
* STAGE: pT3b.
* Immunophenotyping for (PAX-8, P53, ER, ...) is recommended.

Signature:
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايرينى واصف ابراهيم واصف
Analysis ID : [redacted] 1008099
Age : 38 Y : 2 M : 2 D
Gender : Female
Referred By Prof. Dr. : فؤاد عبد الشهيد
Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules
Nature of Specimen : Total abdominal hysterectomy & bilateral salpingeo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration
Receiving Date : 2025-11-11
Patient ID : 743010

PATHOLOGY REPORT

Gross :

Three designated containers were received as:
1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy specimen,
Uterus measuring 6.5x6x3 cm. Endometrial thickness 0.3 cm, posterior outer surface of the uterus showed multiple nodules, the largest measured 2x2x1.5 cm. Myometrial thickness 2 cm and showed trabeculations.
Cervix 4 cm long, 3.5 cm in diameter with rubbery grayish white cut section.
Attached vaginal cuff was 1.5 cm.
RT ovary measuring 3.5x2.5X1 cm, ovarian mass measured 4x2x1.5 cm, cut section was greyish white rubbery causing disruption of ovarian capsule, right tube measured 6 cm long, showed fairly defined mass measured 12x7x4 cm, cut section was varigated greyish white yellowish.
LT ovary measuring 3x2x1 cm, ovarian mass measured 2.5x2x1 cm, cut section was greyish white rubbery causing disruption of the ovarian capsule, left tube measured 7 cm long.
Also received in the same container, Appendicectomy specimen measured 6 cm long, with 0.6 cm in diameter, maximal wall thickness was 0.3 cm, outer surface was glistening showing multiple nodules measured up to 0.8x0.8 cm.
2) Omentectomy: Specimen measured 42x8 cm, showing multiple firm nodules measured up to 1.8x1.8 cm.
Also received in the same container multiple peritoneal nodules measured up to 2x2 cm.
3) 16 cc of reddish fluid.

Microscopic :

Sections examined from the specimen (1) received revealed both ovaries showing infiltrating tumour formed of malignant epithelial cells with moderate to marked nuclear pleomorphism with scattered mitotic activity arranged in solid sheets, acini and papillae.
There is moderate desmoplasia.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ايرينى واصف ابراهيم واصف
Analysis ID : [redacted] 1008099
Age : 38 Y : 2 M : 2 D
Gender : Female
Referred By Prof. Dr. : فؤاد عبد الشهيد
Clinical Diagnosis : Right ovarian mass with massive pelvic peritoneal nodules
Nature of Specimen : Total abdominal hysterectomy & bilateral salpingeo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration
Receiving Date : 2025-11-11
Patient ID : 743010

PATHOLOGY REPORT

There is focal tumoural necrosis.
Ovarian capsule is disrupted.
Lympho- vascular space invasion is present.
Uterine wall revealed simple endometrial hyperplasia.
There is positive adherent peritoneal deposits.
Cervix is free.
Free vaginal cuff. The appendix is involved.
The tumor is adherent to outer surface of both tubes (right and left).
Sections examined from the specimen (2) received revealed omental tissue and peritoneal tissue positive for tumor deposits.
Cytological examination of the films and cell block prepared from the centrifugate of the fluid (3) received revealed eosinophilic proteinaceous material, red blood cells, lymphocytes, mesothelial cells and groups of cells with hyperchromatic nuclei.

Diagnosis :

Right ovarian mass with pelvic peritoneal nodules,
1) Total abdominal hysterectomy & bilateral salpingeo-oophorectomy & Appendicectomy. BILATERAL OVARIAN HIGH GRADE CARCINOMA LIKELY HIGH GRADE SEROUS CARCINOMA, INVOLVING BOTH TUBES FROM OUTSIDE, POSITIVE ADHERENT PERITONEUM, INVOLVED APPENDIX, FREE UTERINE WALL, FREE CERVIX, FREE VAGINAL CUFF.
2) Omentectomy with peritonectomy, POSITIVE FOR TUMOR DEPOSITS.
3) Aspiration, Cytology, POSITIVE FOR MALIGNANCY.
- TUMOR STAGE: pT3b.
- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

This report is electronically signed , any alteration or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	: 13003565
Patient No.	: 28708152302281	Request Date	: 23/11/2025
Path No.	: SR-1685/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right ovarian mass with massive pelvic peritoneal nodules.
Nature of specimen: Referred blocks & slides (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration).

Gross Examination:
Thirty referred blocks labelled (1008099).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; PAX8 clone (MRQ-50), P53 clone (Bp53-11) & ER clone (SP1).

Results:
* IPT (Positive control showed appropriate reactivity):
- PAX-8: Shows diffuse strong positive nuclear staining in the tumor cells.
- P53: Strong diffuse positive staining, Mutant.
- ER: Weak patchy positive staining in tumor cells.

Diagnosis:
Right ovarian mass with massive pelvic peritoneal nodules, Referred blocks (Total abdominal hysterectomy & bilateral salpingo-oophorectomy, Appendicectomy, Omentectomy with peritonectomy & Aspiration):
The immunohistochemical findings (Positive PAX-8, Mutant P53 & Weakly positive ER) are in keeping with high grade serous carcinoma of mullerian origin.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / ايرينى واصف ابراهيم واصف	Request No.	13003565
Patient No.	28708152302281	Request Date	23/11/2025
Path No.	SR-1685/25	Verification Date	09/12/2025
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	38 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]